NEUROFUSION AI | PRELIMINARY AI ANALYSIS REPORT

REPORT ID: REP-20261102-503C56

Automated Multi-Modal AI Results • Stage 1: Pending Clinical Sign-Off

November 02, 2026 • 16:42 UTC

PATIENT INFORMATION	ATTENDING NEUROLOGIST	EXAMINATION DETAILS
ID: PAT-E2E-CFAD30 **Name:** Dorothy Henderson **Age/Gender:** 69 yrs / Female **Status:** Idiopathic Parkinsonism	**Name:** Dr. Clara Rodriguez, MD **License:** MD-NEURO-77491 **Dept:** Neurology & Movement Disorders **Hospital:** Metropolitan Neurological Inst.	**Scan Date:** 2026-11-02 **Report Date:** 2026-11-02 **Modality:** Fusion **Device/Protocol:** NeuroFusionAI Protocol PARKINSON v2.0

PRIMARY AI PREDICTION: NORMAL

Confidence: 100.0%

Target: Parkinson's Disease

Class Probability Distribution: Normal: 100.0% • Parkinson: 0.0%

AI Confidence Calibrated via Temperature Scaling

7. CLINICAL DECISION SUPPORT SYSTEM (CDSS) RECOMMENDATIONS

RISK LEVEL:	Evidence-Based Diagnostic Recommendations:
LOW CLINICAL RISK **Urgency:** Routine Outpatient Follow-up	• Correlate AI automated assessment with comprehensive MDS-UPDRS motor and cognitive exam. • Recommend DaTscan SPECT dopamine transporter neuroimaging to confirm nigrostriatal deficit. • Order formal neuropsychological battery (MoCA / MMSE) to track longitudinal cognitive trajectory. • Initiate outpatient physical therapy and occupational therapy motor rehabilitation.

PATIENT LONGITUDINAL DIAGNOSTIC HISTORY

Date	Modality	Prediction	Confidence	Clinical Status
2026-11-02	Fusion	Normal	100.0%	Verified Match

PRELIMINARY AI SYSTEM OBSERVATIONS:	CLINICAL APPROVAL STATUS
Multimodal attention fusion indicates high concordance between vocal dysphonia and motor tremor.	*Preliminary AI analysis for decision support. Official physician certification pending.* Status: **AWAITING PHYSICIAN REVIEW & APPROVAL**

REGULATORY MEDICAL DISCLAIMER & CLINICAL INTENDED USE:

NeuroFusionAI is an investigational Artificial Intelligence Clinical Decision Support System (CDSS) designed to augment neurological assessment. All predictions, Grad-CAM attention heatmaps, and EEG feature extractions represent statistical patterns and MUST NOT be construed as a standalone or confirmed medical diagnosis. Clinical treatment decisions must be made solely by a board-certified physician in conjunction with the patient's full medical history.